

Fistulas: Genitourinary and Rectovaginal

WARWIKI

Patient Information · An abnormal opening that lets urine, gas or stool leak

A fistula is a small tunnel between two organs that should not be joined. In the pelvis, it can form between the **vagina and the bladder, urethra or ureter**. This causes urine to leak all the time. It can also form between the **rectum and the vagina**. This lets gas or stool pass through the vagina. Most fistulas can be repaired.

About This Condition

- **Urinary fistula** (such as vesicovaginal): **constant urine leaking** from the vagina, day and night.
- **Rectovaginal fistula**: **gas or stool** passing through the vagina, with infections and sore skin.

These symptoms can be upsetting and lonely. They are not caused by anything you did.

What Causes It

- **Pelvic surgery**, such as a hysterectomy. This is the most common cause in the United States and similar countries.
- **Childbirth**, especially a long, blocked labor or a bad tear. This is a leading cause worldwide.
- **Radiation**, Crohn's disease, infection, or, less often, cancer

Results in Numbers

about
9 in 10

simple bladder
fistulas close with
the first repair

about
7 in 10

complex fistulas
close (for example
after radiation or a
failed repair)

1 in 2 to 2 in 3

rectovaginal
fistulas after
childbirth heal on
their own within 3–
6 months

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Fistula

An abnormal tunnel joining two organs.

Vesicovaginal

A fistula between the bladder and vagina.

Rectovaginal

A fistula between the rectum and vagina.

Ureter

The tube that carries urine from the kidney to the bladder.

Stent

A soft tube placed inside the ureter to help it heal.

Dye test

Colored fluid is put in the bladder or rectum to find the leak.

Cystoscopy

A look inside the bladder with a thin camera.

Martius flap

A pad of fatty tissue from the labia, moved in to support a repair.

Can it be fixed? Yes. Most fistulas can be repaired. A small, new bladder fistula sometimes closes with a catheter alone, but most need surgery. Surgery is timed for when the tissue is ready. After other surgery, a simple fistula is often repaired within weeks. After radiation, the wait is usually longer.

How It Is Diagnosed

- An exam, often with a **dye test** to find the leak
- **Cystoscopy** and scans to map the fistula and check the ureters
- Checks for a cause, such as radiation, infection or Crohn's disease

How It Is Treated

- 1 **Ureter leak:** a stent is usually tried first, as soon as possible. It works best when placed early.
- 2 **Catheter trial:** a small, new bladder fistula may close with a catheter left in for a few weeks.
- 3 **Repair:** the tunnel is closed in layers, through the vagina or the belly. A flap of healthy tissue may be added, especially after radiation or a failed repair.
- 4 **Stool diversion:** for some rectovaginal fistulas, stool leaves through a stoma for a time.

After a bladder repair, you usually keep a catheter for about 1–3 weeks. Your team may test that it has healed before removing it.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Pelvic or side pain that is getting worse
- Leaking that comes back after a repair

Go to an emergency department if:

- The catheter stops draining and your belly is full or painful
- Heavy bleeding, such as soaking a pad an hour
- You cannot reach the office and are worried

Three Things to Remember

1. Most fistulas can be repaired. Simple ones close with the first repair about 9 times in 10.
2. A leak from a ureter should be treated early.
3. Pads and skin care help while you wait. Ask your team about comfort.